Name	:	Himani P. Thakkar
Age/Sex	:	(22Y 11M 9D / Female)
Reg. No./Mem Id.	:	**2205011220** / HT221
Reg. Date & Time	:	29/04/2022 13:02
Coll. Date & Time	:	29/04/2022 13:05
Ref. By	:	Dr. Nemi Goriwal (M.S.,FAIS)

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HAEMOGRAM REPORT

(By BECKMAN COULTER UNICEL DXH 800 - USA)
(Specimen: Whole Blood)

Investigation	Result	Unit	Biological Ref. Interval	Method
Haemoglobin	12.7	gm%	12.0 - 15.0	Non Cyanide Oxy Hemoglobin
Blood Cell Indices				
RBC Count	4.66	mill./c.mm	3.8 - 4.8	Electrical Impedance
PCV	37.7	%	36 - 46	Calculated
MCV	**80.7**	fL	83 - 101	Derived From RBC Histogram
MCH	27.3	pg	27 - 32	Calculated
MCHC	33.8	%	31.5 - 34.5	Calculated
RDW	**16.5**	%	11.6 - 14.0	Derived From RBC Histogram
RDW – SD	**46.8**	fL	39 - 46	Derived From RBC Histogram
NRBC	0.0	%	0 - 0.3	Coulter VCS-n Technology
Total WBC Count	5200	/c.mm	4000 - 10000	Electrical Impedance
Differential Count				
Polymorphs	55	%	40 - 80	VCS Technology / Microscopy
Lymphocytes	35	%	20 - 40	VCS Technology / Microscopy
Monocytes	07	%	2 - 10	VCS Technology / Microscopy
Eosinophils	03	%	1 - 6	VCS Technology / Microscopy
Basophils	00	%	0 - 2	VCS Technology / Microscopy
Absolute Count				
Polymorphs	2860	/c.mm	2000 - 7000	Calculated
Lymphocytes	1820	/c.mm	1000 - 3000	Calculated
Monocytes	364	/c.mm	200 - 1000	Calculated
Eosinophils	156	/c.mm	00 - 500	Calculated
Basophils	0	/c.mm	00 - 100	Calculated
Platelet Count	218000	/c.mm	1,50,000 - 4,10,000	Electrical Impedance
MPV	8.0	/c.mm	7.9 - 10.8	Derived From Platelet Histogram

MC-4764

Value Abbreviation: **L = Low, PL = Panic Low, H = High, PH = Panic High**
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Approved On
29/04/2022 14:55

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RBC Impression Normochromic Normocytic.
(MICROSCOPIC EXAMINATION)

WBC Impression Appear normal.
(MICROSCOPIC EXAMINATION)

Platelet Impression Adequate & Normal.
(MICROSCOPIC EXAMINATION)

REFERENCES

(1) Practical Hematology by Dacie & Lewis (12th Edition).
(2) Nathan/Oski, Hematology of Infancy and Childhood,2nd Ed,1981.
(3) UniCel DxH Series with System Manager Software.,B26647AD July 2016,Beckman Coulter, Inc.

---------- End Of Report ----------

Dr. Arpita Shah	**Dr. Harsha Pandya**	**Dr. Anand Parikh**	**Dr. Mayank Joshi**
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Consultant Pathologist	**Consultant Pathologist**	**Consultant Pathologist**	

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MALARIA ANTIGEN DETECTION

(Specimen: Whole Blood)

Investigation	Result	Biological Reference Interval	Method
P.Vivax Antigen	Not Detected	Not Detected	Immunochromatography
P.Falciparum Antigen	Not Detected	Not Detected	Immunochromatography

Comments

Malaria is a protozoan parasitic infection, prevalent in the Tropical & Subtropical areas of the world.
Four species of plasmodium parasites are responsible for malarial infections in humans viz.
P.falciparum, P.vivax, P.ovale & P.malariae.
Falciparum infections are associated with Cerebral malaria and drug resistance
whereas vivax infection is associated with high rate of infectivity and relapse.
Differentiation between P.falciparum and P.vivax is of utmost importance for better patient
management and speedy recovery

---------- End Of Report ----------

Dr. Arpita Shah **M.B.B.S., D.C.P.** **Consultant Pathologist**	**Dr. Harsha Pandya** **D.C.P.,M.D.** **Consultant Pathologist**	**Dr. Anand Parikh** **M.D. (PATH)** **Consultant Pathologist**	**Dr. Mayank Joshi** **Managing Director**

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Name : Himani P. Thakkar
Age/Sex : (22Y 11M 9D / Female)
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BLOOD CHEMISTRY

(Carried out on VITROS 5600, a Fully Automated Integrated Dry Chemistry System – USA)
(Specimen: SERUM)

Investigation	Result	Unit	Biological Reference Interval	Method
Calcium	9.1	mg/dL	8.4 - 10.2	Arsenazo III
S.G.P.T. (ALT)	20.5	U/L	0 - 35	P5P with UV

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VITAMIN B12 LEVEL

(Carried out on VITROS 5600, a Fully Automated Integrated Dry Chemistry System - USA)

(Specimen: Serum)

Investigation	Result	Unit	Biological Reference Interval	Method
Vitamin B12	240.0	pg/mL	239 - 931	ECLIA-Competitive IA

Note:-

Reduced concentrations of vitamin B12 may indicate the presence of vitamin dependent anemia.
Measurement of vitamin B12 is intended to identify and monitor vitamin B12 deficiency.

This can arise from the following;

(1) Vegetarians, Pregnancy specially last trimester.
(2) defect in the secretion of Intrinsic Factor, resulting in inadequate absorption from food (pernicious anemia).
(3) gastrectomy and malabsorption due to surgical resection.
(4) a variety of bacterial or inflammatory diseases affecting the small intestine.
(5) Atrophic gastritis, Intrinsic factor antibodies.
(6) Regional ileitis, resected bowel, Celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria.

Elevated concentrations of vitamin B12 have been associated with pregnancy, the use of oral contraceptives and multivitamins and in myeloproliferative diseases, such as chronic granulocytic leukemia and myelomonocytic leukemia. An elevated concentration of vitamin B12 is not known to cause clinical problems.

Blood collected after Vitamin B-12 injection or taking Oral Supplements containing Vitamin B-12 may interfere with test result.

---------- End Of Report ----------

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25-OH VITAMIN D TOTAL

(Carried out on VITROS 5600, a Fully Automated Integrated Dry Chemistry System - USA)

(Specimen: Serum)

Investigation	Result	Unit	Biological Reference Interval	Method
25 – Hydroxy Vitamin D	30.60	ng/mL	< 20 : Deficient 20 - 29 : Insufficient 30 - 100 : Sufficient Level > 100 : Toxic Level	ECLIA-Competitive IA

Interpretation

Deficient - High risk for developing bone disease
Insufficient - Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient - Optimal concentration for maximal health benefit
Potential intoxication - High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.

Comments

Vitamin D promotes absorption of calcium and phosphorus and mineralization of bones and teeth.
Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany.
Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

Decreased Levels

- Inadequate exposure to sunlight
- Dietary deficiency
- Vitamin D malabsorption
- Severe Hepatocellular disease
- Drugs like Anticonvulsants
- Nephrotic syndrome

Increased levels

- Vitamin D intoxication

---------- End Of Report ----------

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THYROID FUNCTIONS

(Carried out on VITROS 5600, a Fully Automated Integrated Dry Chemistry System - USA)

(Specimen: SERUM)

Investigation	Result	Unit	Biological Reference Interval	Method
S. Thyroid Stimulating Hormone(TSH)	0.74	µIU/mL	0.4001 - 4.049 For Pregnant Women 1 st Trimester : 0.1298 - 3.120 2 nd Trimester : 0.2749 - 2.652 3 rd Trimester : 0.3127 - 2.947	ECLIA-Immunometric IA

Note:

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
2. Recommended test for T3 and T4 is unbound fraction or free levels as it is metabolically active.
3. Physiological rise in Total T3 / T4 levels is seen in pregnancy and in patients on steroid therapy.
4. Free Thyroid hormones are more specific and clinically significant than T3 & T4 so Preferered over routine thyroid screening of T3 & T4.

---------- End Of Report ----------

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Name	:	Himani P. Thakkar
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URINE CHEMISTRY EXAMINATION

(Carried out on Fully Automated Urine Chemistry Analyzer : LABUMAT 2)
(Specimen: Urine (Morning/Random/P.P.)**)**

Investigation	Result	Biological Reference Interval	Method
PHYSICAL EXAMINATION			
Colour	Amber*	Pale Yellow	
Transparency	**S.Turbid**	Clear	
CHEMICAL EXAMINATION			
Urine Glucose	Absent	Absent	GOD POD/Benedict's Test
Ketone*	Absent	Negative	Legals Test/Rothera's Test
Specific Gravity	1.015	1.005 - 1.025	Rapid Strip-Bromothymol Blue Indicator
Reaction (pH)	5.0	4.5 - 8.0	Methyl Red Sodium Salt -Bromothymol Blue
Proteins	Absent	Absent	"Protein Error" of pH/S.S.Acid
Bile Salts	Absent	Absent	Hay's Test
Bile Pigments	Absent	Absent	Fouchet's
Bilirubin*	Absent	Negative	Rapid strip - Reaction with Diazonium Salt
UroBilinogen*	Absent	Negative	Rapid strip - Reaction with Diazonium Salt
Leukocyte*	Absent	Negative	Rapid strip - Reaction with Diazonium Salt
Blood*	Absent	Negative	Oxidation of TMB by Hydroperoxide
Nitrite*	Absent	Negative	Griess Reaction

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29/04/2022 14:51 **Report No : 7 of 7**

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URINE MICROSCOPIC EXAMINATION

(Carried out on Fully Automated Urine Analyzer : Urised 3 Pro)
(Specimen: Urine (Morning/Random/P.P.)**)**

Investigation	Result	Unit	Biological Reference Interval
Pus Cells			
Pus Cells (Normal)	0.30	/HPF	0 - 5
Pus Cells (Clumps)	0.00	/HPF	
Red Cells			
Red Cells (Normal)	**2.30**	/HPF	0 - 2
Red Cells (Dismorphic)	0.00	/HPF	
Red Cells (Crenated)	0.00	/HPF	
Epithelial Cells			
Epi.Cells (Squamous)	2.50	/HPF	0 - 15
Epi.Cells (Non Squamous)	0.30	/HPF	0 - 1
Casts			
Hyaline Cast	0.00	/HPF	0 - 5
Granular Cast	0.00	/HPF	Absent
RBC Cast	0.00	/HPF	Absent
Crystals			
Ca.Ox. (Monohydrate)	0.00	/HPF	Absent
Ca.Ox. (Dihydrate)	0.00	/HPF	Absent
Triple-Phosphate	0.00	/HPF	Absent
Uric Acid	0.00	/HPF	Absent
Amorphous	**0.80**	/HPF	Absent

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